

INCLUSION OF INDIGENOUS PEOPLE IN THE PROVISION OF HEALTH SERVICES IN SAN LUIS POTOSÍ MEXICO.

Idalia Rojas Martínez . (COPOCYT)

Advisor: Alejandro Robinson-Cortes. (University of Exeter)

Introduction

Inclusion refers to providing care in a dignified and high-quality manner, with an intercultural approach according to the needs of the individuals, families, and communities. According to Aguirre Beltrán (2011), an intercultural approach to face diseases is based on a mix of healthcare traditions and institutions. The model for Intercultural Healthcare of Indigenous and African American Peoples considers human, cultural, and linguistic rights as fundamental parts of healthcare, and emphasizes the low access that indigenous populations have to healthcare services.

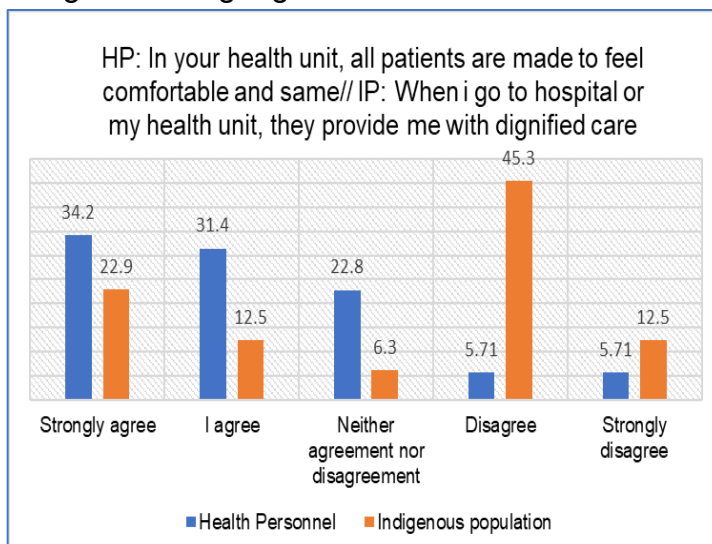
Mocellin-Raymundo, M., Viesca-Treviño, C., & Gutiérrez-Martínez, D. (2011). Bioethics and intercultural health: notes for a necessary and possible connection. Medical Journal of the Mexican Institute of Social Security 49. <https://www.redalyc.org/articulo.oa?id=457745502017>

Motivation

In Mexico there are deficiencies in the provision of health services to the indigenous population. For instance, there does not exist a public record of the number of nurse staff who belong to an indigenous group or speak an indigenous language. Data from a CONEVAL report (2023) indicates that in San Luis Potosí there are 250,620 indigenous people, and, within this group, 44.6% live in extreme poverty.

Results

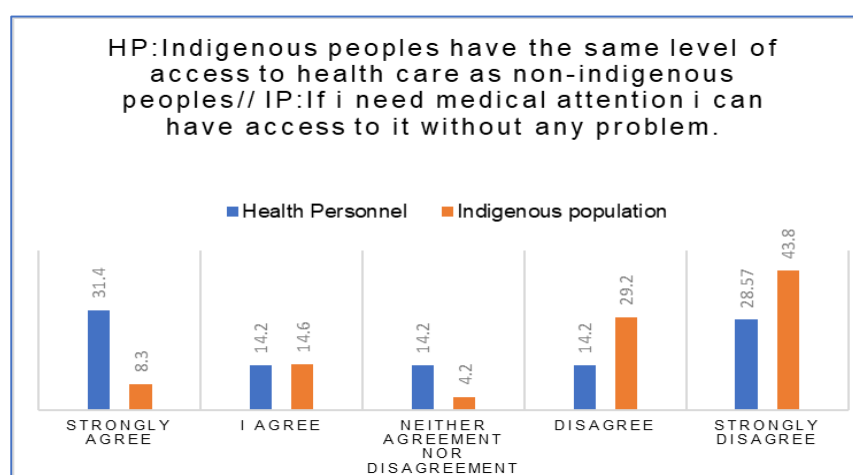
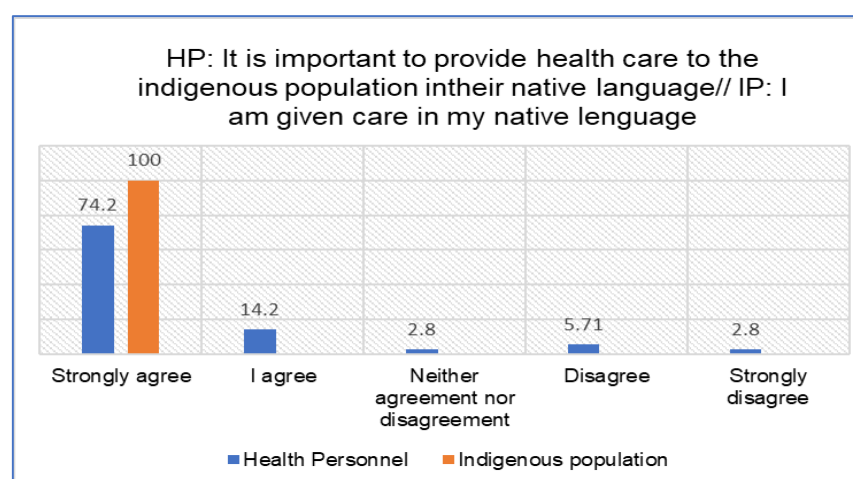
Our first pilot survey was conducted during three days (October 7–9th, 2023) in the aforementioned four communities. Surveys were conducted at community centers and door to door. Our response rate was 83%. In total, we gathered responses from 48 survey participants across the four communities. 80% of survey respondents were women, and 20% men. 60% of them reported having been discriminated against or excluded when attending a healthcare center. A common trend among respondents was their view that the health personnel do not adapt to their needs (e.g., they receive prescriptions but have no access to medicines, or have trouble communicating effectively with health personnel who do not speak an indigenous language). On average, among survey participants, the time of travel to their closest health unit is approximately 2-3 hours. Despite health units being in indigenous communities, they tend to lack health personnel who speak an indigenous language.



Objective

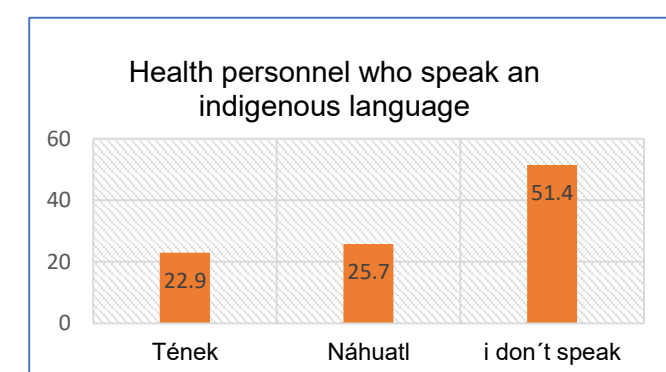
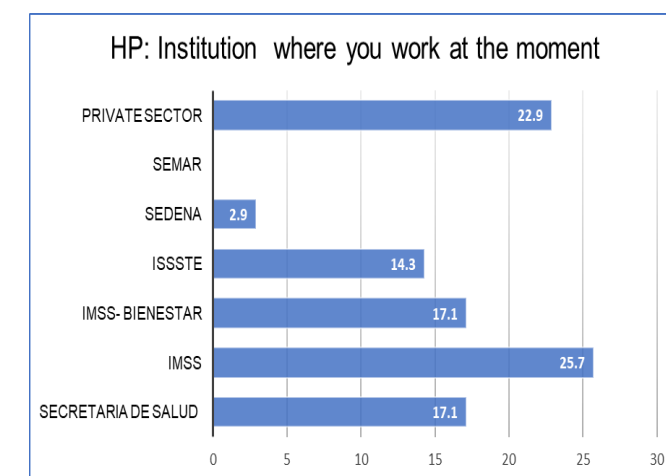
Identify the degree of inclusion for indigenous-speaking populations in the provision of health services in San Luis Potosí, Mexico.

From our second survey, made to healthcare professionals via WhatsApp, we gathered 35 responses, 34 from nurse staff and 1 medical doctor. Respondents were spread across the four areas of San Luis Potosí: Huasteca: 54.3%; Center: 37.4%; Middle 5.7%, and Highlands 2.9%. The minimum educational level is nursing technician (8.6%) and the maximum is doctorate (5.7%). 48% of health professionals recognize that indigenous peoples have less access to health. In terms of health promotion material in an indigenous language, 42% mention that they do not prepare it, and 25% say they do not possess the sufficient knowledge to do so.



Methodology

Two pilot surveys were conducted: one to members of indigenous communities, and another to healthcare professionals. Both were run using convenience sampling. The first one was conducted in-person in four indigenous communities of the Huasteca Potosina in San Luis Potosí, Mexico: San José Pequetzén, Cruztubub, El Aguacate, and Tanchahuil. The second one was conducted online (Google form distributed through WhatsApp) to healthcare professionals. Both survey questionnaires followed the instrument "Index for inclusion" (Booth, Ainscow, and Kingston, 1999), validated and applied in various schools in the United Kingdom during 1997-1998, with a reliability of 0.81 Cronbach's Alpha. Our modified version obtained a Cronbach's Alpha of 0.71. Both of our questionnaires consist of 17 statements that evaluate health personnel across several dimensions (e.g., quality of care, the use of teaching material in an indigenous language, and the accessibility of healthcare services to indigenous populations). It uses a five-point Likert scale with a score of 1 (totally agree) to 5 (totally disagree).



Conclusions.

Although there are healthcare models with an intercultural approach and a high fraction of health professionals knows about intercultural issues, the provision of healthcare to indigenous population is not adequate. Among others, because healthcare professionals do not have the necessary material and supplies, or count with the adequate support and personnel. Indigenous peoples are disproportionately affected by this problem since not only their language is a barrier to care, but also the lack of supplies, the distance to their health units, and that, on some occasions, they feel excluded from the healthcare system.

We thank all the participants who made this project possible, especially the indigenous communities and health services.